

Incident Reporting and Event Severity Classification

Policy Number	POL-RM-013	Owner / Department	Risk Management / Patient Safety
Effective Date	2024-01-15	Version	4.2
Last Revised	2026-02-10	Next Review	2028-02-10
Approved By	Patient Safety & Quality Committee	Applies To	All staff, licensed providers, students, and contractors, all departments and units

1. Purpose

This policy establishes a single, facility-wide process for reporting patient-safety events, staff-safety events, and near misses, and a common severity scale used to triage, escalate, and notify. It is the governing reference for event severity and notification timing; topic-specific policies restate these rules for their domain and must not conflict with this document.

2. Scope

Applies to any event or unsafe condition that reached a patient, visitor, or staff member, as well as near misses (events intercepted before reaching a patient) and unsafe conditions. Reporting is non-punitive; the facility maintains a just-culture framework and does not discipline staff for good-faith reporting of errors or near misses.

3. Definitions

- Event** — any happening not consistent with the routine operation of the facility or routine care of a patient.
- Near miss (close call)** — an event that did not reach the patient, whether by chance or active interception.
- Harm** — physical, psychological, or functional injury. Graded as none, mild/temporary, moderate/temporary, severe/temporary, permanent, or death.
- Sentinel event** — a patient-safety event that reaches a patient and results in death, permanent harm, or severe temporary harm (The Joint Commission). All sentinel events are SEV-1.
- Serious Reportable Event (SRE / “never event”)** — one of the 29 NQF events in 7 categories (e.g., wrong-site surgery, retained foreign object, air embolism, Stage 3/4 hospital-acquired pressure injury).

4. Policy — Severity Classification

Every reported event is assigned one severity level. When the facts could support two levels, assign the higher level and let review adjust it. Severity drives the notification and review requirements in Section 6.

Level	Definition	Typical examples
SEV-1 Catastrophic / Sentinel	Death, permanent harm, or severe temporary harm; any sentinel event or Serious Reportable Event; any event requiring a life-sustaining intervention.	Wrong-site surgery; hemolytic transfusion reaction; fall with intracranial hemorrhage; suicide of an inpatient; infant abduction.

Level	Definition	Typical examples
SEV-2 Major	Temporary harm requiring intervention, treatment, or an increased level of care, or a prolonged length of stay.	Medication error reaching the patient and requiring treatment; fall with fracture; Stage 2 hospital-acquired pressure injury.
SEV-3 Minor	Event reached the patient but caused no harm or only minimal harm not requiring treatment beyond monitoring.	Medication given one hour late without consequence; unwitnessed fall with no injury; mislabeled non-critical specimen caught before resulting.
Near Miss	Error or unsafe condition intercepted before reaching the patient.	Wrong medication caught at the bedside scan; incorrect blood unit caught during the two-person check.

5. Procedure — Filing a Report

1. Ensure the patient and the area are safe; render immediate clinical care before documenting.
2. File an event report in the electronic reporting system as soon as the situation is stable, and no later than the end of the shift on which the event occurred or was discovered.
3. Describe the facts objectively — what happened, when, where, who was involved, and what was done. Do not record blame, opinion, or speculation.
4. Record the patient's condition and any monitoring or treatment initiated in response.
5. Notify per the matrix in Section 6 based on the assigned severity. When in doubt about severity, escalate.

IMPORTANT — The event report is a confidential quality-improvement document. Do not file it in or reference it from the patient's medical record. Clinical facts belong in the medical record; the event report does not.

6. Reporting & Notification Requirements

Severity	Who must be notified	By when	Review
SEV-1	Attending provider, Charge Nurse, Nursing Supervisor, Risk Management, and the Administrator-on-Call.	Immediately, and never later than 1 hour from discovery.	Mandatory human review. Comprehensive systematic analysis (RCA ²) and action plan within 45 business days for sentinel events.
SEV-2	Attending provider, Charge Nurse, and Risk Management.	Within 4 hours.	Human review within 3 business days.
SEV-3	Charge Nurse; Risk Management via the event report.	Within 24 hours.	Quality review in aggregate.
Near Miss	Charge Nurse; Risk Management via the event report.	Within 24 hours.	Aggregate trend review.

External reporting (State Department of Health, CMS, FDA MedWatch, or accrediting bodies) is coordinated solely by Risk Management and is not initiated at the unit level.

7. Documentation Requirements

- Reporter name, role, and contact; date/time of the event and of discovery.
- Location (unit/room) and persons involved (patient MRN, staff, witnesses).
- Objective description of the event and immediate actions taken.
- Patient condition before and after; monitoring and treatment initiated.
- Assigned severity and every notification made (who, when, by what method).

8. References

- The Joint Commission, Sentinel Event Policy (CAMH), comprehensive systematic analysis within 45 business days.
- National Quality Forum, Serious Reportable Events (2011) — 29 events in 7 categories.
- AHRQ Common Formats for Event Reporting.
- Related: POL-EH-001, POL-PS-002, POL-PS-003, POL-RM-004, POL-LAB-006, POL-SURG-007, POL-PS-008, POL-NUR-009.